



NEVADA
RURAL HEALTH TRANSFORMATION



CMS Programmatic Annual Report

Basic and General Walkthrough

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September 8, 2026

Agenda

Purpose: One sentence describing the goal of this presentation and what the audience will take away.

1 Basics and Timelines

2 Initiative Reporting

3 State Policy Actions (SPA)

4 Obligated and Spent Funds Reporting

Reporting Basics and Timelines

- NVHA is responsible for both Quarterly and Annual Reports.

Q1: Aug. – Oct. (Due Nov. 29)

Q2: Nov. – Jan. (Due March 1)

Q3: Feb. – April (Due May 31)

Annual/Q4: May – July (Due Aug. 31)

- Annual Reporting Template was approved by OMB on August 14th with a due date of August 31st. Since the release, CMS held multiple technical assistance webinars to answer the many questions they received.
- The reporting period for the first annual report ended July 31, 2026.
- Key Terms: Stages, Checkpoints, Milestones, Initiative Information Report (IIR) Metrics

Initiative 1: Rural Outcome Accelerator Program

Number of People Served		
e provide the number of people served		0
Metrics		
Metric	Current Value	As of Date
Percentage of rural adults ages 18 to 85 who had a diagnosis of hypertension and whose blood pressure (BP) was adequately controlled (<140/90 mm Hg).	32.43%	12/31/2025
Percentage of deliveries that received a prenatal care visit in the first trimester on or before the enrollment start date or within 42 days of enrollment in the organization.	47.73%	12/31/2025
Percentage of deliveries that had a postpartum visit on or between 7 and 84 days after delivery.	56.34%	12/31/2025
Percentage of ED visits for persons 6 years of age or older with a principal diagnosis of mental illness, or any diagnosis of intentional self-harm, that had a mental health follow-up service within 30 days of the	55.07%	12/31/2025
Percentage of ED visits for persons 13 years of age or older with a principal diagnosis of SUD, or any diagnosis of drug overdose, for which there was a follow-up service within 30 days of the ED visit.	33.96%	12/31/2025

Stage 0 – Planning		
Checkpoint	Yes/No	Attachment(s)
Establish governance	Yes	Attachment 0.1 Governance
Submit project plan to CMS	Yes	Attachment 1.1 Project Plan
Stage 1 – Project Preparation		
Checkpoint	Yes/No	Attachment(s)
CMS approval of project plan	Yes	Attachment 1.1 Project Plan
Launch initiative	No	

CMS Review (CMS Use Only)	
Stage 0 - Planning	
CMS Status Evaluation	Comments
Stage 1 - Project Preparation	
CMS Status Evaluation	Comments

Initiative 2: NV Rural Health System Flex Fund

Metrics			
#	Metric	Current Value	As of Date
1a	Utilization rates by service type for rural counties for nonemergency transport services in rural counties. (Unique recipients/Total rural Medicaid enrollees)	Recipients: 2,407 Rural MA Enrollees: 73,515 3.3%	CY2025
1b	Utilization rates by service type for rural counties for emergency transport services in rural counties. (Unique recipients/Total rural Medicaid enrollees)	Recipients: 5,373 Rural MA Enrollees: 73,515 7.3%	CY2025
2	Updated or replaced equipment and infrastructure for rural Nevada healthcare providers	First Reported Value to be Used as Baseline	7/31/2026
3	Number of mobile care units serving rural counties	First Reported Value to be Used as Baseline	7/31/2026
4	Median days cash on hand as (Cash on Hand and in Banks + Temporary Investments + Investments) / (Operating Expense - Depreciation Cost) / 365)	First Reported Value to be Used as Baseline	7/31/2026

Stage 0 – Planning			
	Checkpoint	Yes/No	Attachment(s)
0.1	Establish governance	Yes	Attachment 0.1 Governance
0.2	Submit project plan to CMS	Yes	Attachment 1.1 Project Plan
Stage 1 – Project Preparation			
	Checkpoint	Yes/No	Attachment(s)
1.1	CMS approval of project plan	Yes	Attachment 1.1 Project Plan
1.2	Launch initiative	Yes	Attachment 1.2 Launch Initiatives

CMS Review (CMS Use Only)	
Stage 0 - Planning	
CMS Status Evaluation	Comments
Stage 1 - Project Preparation	
CMS Status Evaluation	Comments

Initiative 3: Workforce Recruitment and Rural Access Program

#	Metric	Current Value	As of Date		
1	Number of primary care physicians serving rural Nevada	190	7/1/2026		
2	Number of advanced practice nurses in rural Nevada	155	7/1/2026		
3	Number of physician assistants serving in rural	85	7/1/2026		
4	Number of registered nurses in rural Nevada	1906	7/1/2026		
5	Number of behavioral health providers in rural Nevada.	285	7/1/2026		
Stage 0 – Planning				CMS Review (CMS Use Only)	
Stage 0 – Planning				Stage 0 - Planning	
Checkpoint		Yes/No	Attachment(s)	CMS Status Evaluation	Comments
0.1	Establish governance	Yes	Attachment 0.1 Governance		
0.2	Submit project plan to CMS	Yes	Attachment 1.1 Project Plan		
Stage 1 – Project Preparation				Stage 1 - Project Preparation	
Checkpoint		Yes/No	Attachment(s)	CMS Status Evaluation	Comments
1.1	CMS approval of project plan	Yes	Attachment 1.1 Project Plan		
1.2	Launch initiative	Yes	Attachment 1.2 Launch Initiatives		

Initiative 4: Rural Health Innovation & Technology Grant

Metrics			
#	Metric	Current Value	As of Date
1	Measure year over year percentage change of rural Medicaid claims submitted for telehealth services stratified by behavioral and physical health.	4,767 Behavioral Health 39,900 Physical Health Total: 44,667	7/1/2026
2	Measure the year over year percentage change of rural Medicaid providers that participate in CMS Health technology ecosystem, including: 1) Engaging in interoperable data exchange with other providers (to perform key functions such as care management and coordination, referral management); and/or 2) Give rural beneficiaries access to their personal health record; and/or 3) Are connected to an health information exchange.	First Reported Value to be Used as Baseline	7/31/2026
3	Number of rural Medicaid providers that: 1) Completing security risk assessments; or 2) Implementing mitigation strategies; or 3) Implementing and maintaining cybersecurity capabilities.	First Reported Value to be Used as Baseline	7/31/2026
4	Percentage of rural providers implementing and actively using advanced health technologies, including AI-enabled tools, for clinical care, remote monitoring, diagnostics, or administrative workflows.	First Reported Value to be Used as Baseline	7/31/2026

Stage 0 – Planning			
Checkpoint		Yes/No	Attachment(s)
0.1	Establish governance	Yes	Attachment 0.1 Governance
0.2	Submit project plan to CMS	Yes	Attachment 1.1 Project Plan
Stage 1 – Project Preparation			
Checkpoint		Yes/No	Attachment(s)
1.1	CMS approval of project plan	Yes	Attachment 0.2 Project Plan
1.2	Launch initiative	No	

CMS Review (CMS Use Only)	
Stage 0 - Planning	
CMS Status Evaluation	Comments
Stage 1 - Project Preparation	
CMS Status Evaluation	Comments

SPA: State Policy Actions

Presidential Fitness Test

- 100 pts. for states that reestablish the Presidential Fitness Test
- NVHA anticipates activity towards this SPA in BP2

SNAP Food Restriction Waiver Policy

- 100 pts. for USDA approved waiver prohibiting the purchase of non-nutritious items in SNAP
- NV Dept. of Social Services received federal approval effective Feb. 1, 2026

Nutrition CME

- 100 pts. for requiring nutrition CME for physicians
- BDR introduced by a state legislator during the Nevada's biannual legislative session.

Physician Assistant Compact

- 100 pts. for Legislation enacted to become a PA Compact member
- BDR introduced by a state legislator during the Nevada's biannual legislative session.

First-Tier Entities Tab

				Reference Columns						
Entity ID	First-Tier Entity (Subrecipient or Contractor)	Entity Type	Obligated Funds	Uncategorized Funds	Total Disbursed Funds	Unliquidated Obligations	Sum of Downstream Disbursements	Retained Disbursed Funds	Sum of Unpaid Downstream Commitments	Net Available Unliquidated Obligations
				Obligated Funds – Sum of Obligations assigned on Use of Funds tab	Sum of Disbursed Amount assigned on Use of Funds tab	Column E – Column G	Sum of Column J across second-tier entities on Downstream Reporting tab	Column G – Column I	Sum of Column K across all downstream entities on Downstream Reporting tab	Column H – Column K
1	Health Services Advisory Group	Technical Assistance Provider	\$429,574.00	\$0.00	\$107,394.00	\$322,180.00	\$0.00	\$107,394.00	\$0.00	\$322,180.00
2	Milliman Inc	Technical Assistance Provider	\$468,440.00	\$0.00	\$426,440.00	\$42,000.00	\$0.00	\$426,440.00	\$0.00	\$42,000.00
3	Health Management Associates	Technical Assistance Provider	\$538,000.00	\$0.00	\$445,000.00	\$93,000.00	\$0.00	\$445,000.00	\$0.00	\$93,000.00
4	Manatt Health Strategies	Technical Assistance Provider	\$395,000.00	\$0.00	\$150,288.50	\$244,711.50	\$0.00	\$150,288.50	\$0.00	\$244,711.50
5	Reliable Health Care Services	Other	\$144,748.88	\$0.00	\$69,537.36	\$75,211.52	\$0.00	\$69,537.36	\$0.00	\$75,211.52
6	State of Nevada	Other State Agency	\$400,783.38	\$0.00	\$400,783.38	\$0.00	\$0.00	\$400,783.38	\$0.00	\$0.00

Use of Funds Tab

							Funding Amounts of Program-Specific Limitations (\$)					
Obligation ID	Obligation Amount	Disbursed Amount	First-Tier Entity (Subrecipient or Contractor)	Description	Initiative Number	Use of Funds Category	Administrative Costs	Capital Expenditures and Infrastructure	Provider Payments	EMR Replacement	Rural Tech Catalyst Fund	Confirmation: No Program Specific Limitation Applies
NV5	\$42,000.00	\$0.00	Health Management Associates	Provide resources, support & technical assistance for feasibility reviews.	4	N/A (Program Administration Expense)	\$42,000.00					
NV6	\$395,000.00	\$150,288.50	Manatt Health Strategies	Traditional Tribal Healthcare Practices 1115 Waiver Support	All	N/A (Program Administration Expense)	\$395,000.00					
NV7	\$144,748.88	\$69,537.36	Reliable Health Care Services	Provide two contract staff to support RHT program at NVHA.	All	N/A (Program Administration Expense)	\$144,748.88					
NV8	\$340,133.68	\$340,133.68	State of Nevada	RHT Program Personnel Costs	All	N/A (Program Administration Expense)	\$340,133.68					
NV9	\$11,173.90	\$11,173.90	State of Nevada	RHT Program Travel Costs	All	N/A (Program Administration Expense)	\$11,173.90					
NV10	\$49,475.80	\$49,475.80	State of Nevada	RHT Program Supplies and Other Costs	All	N/A (Program Administration Expense)	\$49,475.80					
NV11												

Downstream Reporting Tab

FROM		TO					Reference Columns			
Parent ID	Parent Entity	Entity ID	Downstream Entity (Subrecipient or Contractor)	Entity Type	RHTP \$ Committed to Downstream Entity	Disbursed Funds Retained by Downstream Entity	Sum of Downstream Disbursements	Total Disbursed Funds Accounted For	Sum of Unpaid Downstream Commitments	Net Unpaid Committed Funds
							<i>Sum of Column H across all downstream entities</i>	<i>Column H + Column I</i>	<i>Sum of Column G across immediately downstream entities – Column I</i>	<i>Column G – Column J – Column K</i>

RHTP Spending Caps

- **Administrative Costs: 10% - \$17,993,160**
- **Provider Payments (Category B): 15% - \$26,989,741**
- **Capital Expenditure & Infrastructure (Category J): 20% - \$35,986,321**
- **Electronic Medical Record Replacement: 5% - \$8,996,580**
- **Rural Technology Catalyst Fund: 10% or \$20M (Whichever is less)**

Thank you for your participation!

Learn More & Stay Involved



Website: www.nvha.nv.gov/RHTP/

Contact Us

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Questions ?